

REPORT

REDUCTION IN DEMAND FOR ILLICIT DRUG AND ILLICIT DEMAND FOR LICIT DRUGS IN PAKISTAN

An Urgent Need for Collaboration between the Ministry of Health and Antinarcotics Bureau

PROFESSOR INAYAT KHAN

*Former Chief Medical Officer
WHO Headquarters, Geneva*

I. It was on 17 December 1997 that I was interviewed on PTV in their early morning programme (Sawera Sawera) regarding drug control in Pakistan. The interviewer abruptly asked me to talk on the subject as if I was addressing the Prime Minister of Pakistan. Most of the time I covered drug production, drug control and, in particular, post marketing surveillance of drugs. He also asked me about the drug abuse situation in Pakistan. My reply was that it has got two sides, one is reduction in supply and second" reduction in demand. It is the law enforcement agents who are primarily responsible for reducing or stopping the availability of illicit drugs to the population. This group also has a vital role in preventing availability of certain licit drugs liable to produce addiction without a medical prescription. It is in this area that they have to cooperate with the health ministry and the health care professionals. The reduction in demand is a more complex issue and varies from preventive education, early diagnosis, interventions, and treatment facilities, epidemiological investigations and adequate rehabilitation facilities. These activities are primarily the responsibility of the health ministry and health care professionals. The Antinarcotics Ministry has to assist in these activities with the Ministry of Health and particularly with sharing their resources available to them from abroad.

In my interview with the PTV I give an opinion that while activities in reduction in

supply were going on adequately, the reduction in demand activities were inadequate. I did not elaborate this issue further due to shortage of time.

II. I would like to elaborate on the issues of reduction in demand for drugs and where the antinarcotic bureau has failed to cooperate with the Ministry of Health and the health care professionals in Pakistan. This is my impression and I hope that someone can prove me wrong by giving me facts and figures.

I will give a historical profile to the governmental efforts in this area. It was in 1974 that the Pakistan Narcotics Control Board (PNCB) was established to reduce the supply of opium and cannabis which was being smuggled abroad to Europe and America. It was primarily a law enforcement organ set up under a capable police officer, Mr. Yusouf Orakzai. He did include in his plans some activities in the area of treatment facilities and epidemiological studies. In 1977, I studied the subject of drug abuse policy in Pakistan, along with K.A. Wadud, which was published in the Bulletin on Narcotics Drugs Vol. XXIX, no. 4 2 1-40 (1977). I was an officer at that time responsible for the WHO Programme related to the International Drug Control Treaties between 1976 and 1990. I had an opportunity to work closely with four successive chairmen of PNCB during that period and even afterwards by repeated yearly visits to Islamabad after my separation from

WHO. In 1982 I had proposed to the Planning Commission of Pakistan to establish a National Institute of Drug Dependence covering epidemiological services, preventive activities, treatment and rehabilitation under the joint control of the Ministry of Health and the Narcotics Control Ministry. This plan had been included in the Pakistani Five year plans and has been repeated since then as an exercise in the air.

III. It was during the earlier days of PNCB existence that epidemiological studies were conducted in collaboration with WHO Headquarters and with involvement with the Ministry of Health in addition to PNCB. It was in 1982 that the links between the PNCB and the Ministry of Health and WHO were severed and PNCB, in collaboration with UNDP Islamabad, dealt directly with the Vienna based UNFDAC. This included both the reduction in demand activities as well as the reduction in supply. At present the Antinarcotics Bureau deals directly with the United Nation Drug Control Programme (UNDCP) based in Vienna in both areas of supply and demand with no input from the Ministry of Health. It is the Ministry of Health which deals with WHO both at headquarters in Geneva and at EMRO in Cairo. It is within the domain of WHO that reduction in demand activities fall and thus support Governments throughout the world. UNDCP, International Narcotics Control Board (INCB), and the United Nations Commission on Narcotic Drugs are based in Vienna where I had the pleasure of representing WHO during my services as well as the United Nations General Assembly, which met in February 1990 and had a special session on drug abuse control. Every year the World Health Assembly meet invariably in Geneva and has on its agenda on many occasions drug abuse control and in particular reduction in demand activities. I can say that officials from the Ministry of Health have never been given an opportunity to attend meetings held in Vienna or even in the United Nations in New York, on the other hand, no officials from the Antinarcotics Bureau have participated in the World Health Assembly

when drug abuse was occasionally discussed. I have followed this situation until today and the situation remains the same. This is contrary to the practice followed by most other countries. It was the late Rauf Ali Khan, the former Chairman of PNCB, and later Chairman of INCB, who prepared a plan in 1989 that the structure of PNCB be modified to allow a prominent place for reduction in demand activities and to be specifically handled by technical staff. It was only in 1995 that an officer was assigned to a position created as head of reduction in demand for drugs. But unfortunately he was an administrator and had to learn the ABC of the subject.

IV. In my many visits to Islamabad I twice had discussions with the late President Ziaul Haq, and with President Ghulam Ishaq Khan, that the Narcotics Control Ministry and Health Ministry must start working closely together. The Ministry of Health has the expertise in reduction in demand activities and liaise with doctors, pharmacists, dentists and nurses. While the Antinarcotics Bureau have funds coming from abroad which primarily go to law enforcement activities, the Ministry of Health may be able to convince the Antinarcotics Bureau officials to prepare a paper as to the actual situation in reduction in demand for illicit drugs and illicit demand for licit drugs. This paper can consist of the following items:

1. Epidemiological studies
2. Primary prevention, involving schools and colleges and the public at large. It is welcome that the syllabi up to the 6th grade have been modified and include information on the subject.
3. Early diagnostic facilities.
4. Examples of intervention efforts and their success.
5. Treatment techniques and availability of these facilities.
6. Rehabilitative facilities.
7. The role of health care professionals, social workers, and even law enforcement agents in educational efforts to prevent drug abuse and promote rational use of psychoactive drugs.

IV. This study can be done by a joint effort between the Ministry of Health and Antinarcotics Bureau at the federal level, provincial level, and regional offices. Private sector must also collaborate, in particular professional societies, and non-governmental organizations. Input has to be made from WHO, UNDCP and INCB. Participation of the Pakistan Psychiatric Society, Pakistan Medical Association, and pharmaceutical and nursing professions must be involved.

V. DRUG ABUSE MASTER PLAN FOR PAKISTAN 1998-2003

This document was prepared by the Narcotics Control Division, Government of Pakistan, with the assistance of the United Nations International Drug Control Programme (UNDCP) which is located in Vienna.

From the very beginning this plan states that it has been prepared by involving the provincial governments, the nongovernmental Organizations and the private sector. The recommendations regarding drug demand reduction, interdictions, prosecuting traffickers and reduction of supply of drugs have been made in the backdrop of Pakistan Social Economic Conditions. They hope the plan will go a long way in achieving a drug free society. The Secretary of the Narcotics Control Divisions has thanked those who helped in preparing and updating this document.

I presume that these two classes of health officials were excluded purposely from this study although they along with the health care professionals are the key elements in achieving a drug free Pakistan as it relates to reduction in demand of these substances. The World Health Organization, a key agency, was also ignored.

UNDCP in Vienna has hardly any facilities in reduction in demand and they depend entirely on the World Health Organization, other UN agencies and national participants.

VI. I had a brief private visit to Peshawar in

December 2003-January 2004 with very limited time, but I met the Health Secretary in the NWFP and General Aslam, Director General of Health at Islamabad. I also met Brigadier Inam, Regional Director ANF along with Mr. Fazel Habib at their Peshawar office. I also visited L.R. Hospital and Hayet Shahid Hospital at Peshawar and had discussion with psychiatrists.

VII. I met Professor Khalid Mufti, Head of the Department of Psychiatry and Principal of Khyber Medical College, Peshawar, and convinced him and the PTV Peshawar to have a one-hour discussion on the drug abuse control situation in Pakistan, in Pushto. We also had Brigadier Inam of ANF to participate in these discussions on television. I will refer out of this discussion to one of my urgent statements to be considered by the Government of Pakistan. This is to establish a national institute of drug dependence covering epidemiological services, preventive services and treatment and rehabilitation services under the joint control of the Ministry of Health collaboration with the Anti-Narcotics Ministry and a university. I now wish to add one more point which seems to be urgent and that is the involvement of the Pakistan Medical and Dental Council in the activities of the plan. This is needed to ensure that a standard level of service is provided to patients who are treated by dozens of centres spread all over the country without any existing infrastructure.